



LABORATORY MEDICINE PRO

DEPARTMENT OF PATHOLOGY

Surgical Pathology Consultation Report

Patient Name:		Service:		Accession #:	
MRN:		Visit #:		Collected:	
DOB:				Received:	
Gender:		Location:		Reported:	
HCN:		Facility:			
Ordering MD:					
Copy To:					

Specimen(s) Received

1. Thyroid: RIGHT HEMITHYROIDECTOMY STITCH UPPER POLE

Diagnosis

Multifocal papillary carcinoma, dominant 5.1 cm, locally invasive with prominent cribriform architecture: Thyroid
 Metastatic papillary thyroid carcinoma: Lymph nodes, 3, perithyroidal
 No pathological diagnosis: Thymus
 Right hemithyroidectomy specimen

Comment

This is a large locally invasive papillary carcinoma with striking cribriform architecture. No morules are identified. The cribriform morular variant of papillary carcinoma is usually associated with familial polyposis coli and a germline APC mutation; in this case, the full features of this variant are not identified but the prominent and unusual cribriform architecture suggest that further investigation for an underlying genetic predisposition may be warranted.

Synoptic Data

Procedure:	Right hemithyroidectomy
Received:	Fresh
Specimen Integrity:	Intact
Specimen Size:	Right lobe: 6.8 cm 4.4 cm 3.6 cm Isthmus +/- pyramidal lobe: 2.2 cm 0.7 cm 0.8 cm
Specimen Weight:	52.9 g
Tumor Focality:	Multifocal, ipsilateral Multifocal, midline (isthmus)
----- DOMINANT TUMOR -----	
Tumor Laterality:	Right lobe
Tumor Size:	Greatest dimension: 5.1cm Additional dimension: 4.3 cm Additional dimension: 3.5 cm
Histologic Type:	Papillary carcinoma, other variant: Cribriform without morules

ICD-0-3
 (w/ cribriform, nos 8201) Code to highest
 carcinoma, papillary, thyroid 8260/3
 Site: thyroid, nos 673.9
 dw
 10/2/12

Surgical Pathology Consultation Report

Histologic Grade:	Cribriform-morular architecture
Margins:	Classical cytomorphology
Tumor Capsule:	Not applicable
Tumor Capsular Invasion:	Margins uninvolved by carcinoma
Lymph-Vascular Invasion:	Partially encapsulated
Perineural Invasion:	Present, extent minimal
Extrathyroidal Extension:	Not identified
----- SECOND TUMOR -----	
Tumor Laterality:	Isthmus
Tumor Size:	Greatest dimension: 0.2 cm
	Additional dimension: 0.2 cm
Histologic Type:	Papillary carcinoma, follicular variant
	Papillary carcinoma, microcarcinoma (occult, small or microscopic) variant
	Follicular architecture
	Classical cytomorphology
Histologic Grade:	Not applicable
Margins:	Margins uninvolved by carcinoma
Tumor Capsule:	None
Tumor Capsular Invasion:	Not identified
Lymph-Vascular Invasion:	Not identified
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Not identified
TNM Descriptors:	m (multiple primary tumors)
Primary Tumor (pT):	pT3: Tumor more than 4 cm, limited to thyroid or with minimal extrathyroidal extension (eg, extension to sternothyroid muscle or perithyroid soft tissues)
Regional Lymph Nodes (pN):	pN1a: Nodal metastases to Level VI (pretracheal, paratracheal and prelaryngeal/Delphian) lymph nodes
	Number of regional lymph nodes examined: 3
	Number of regional lymph nodes involved: 3
	Lymph Node, Extranodal Extension Not identified
Distant Metastasis (pM):	Not applicable
Additional Pathologic Findings:	None Identified

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Electronically verified by:

Gross Description

The specimen labeled with the patient's name and as "right hemithyroidectomy stitch upper pole" consists of a right hemithyroidectomy specimen that is oriented with a suture and weighs 52.9 g. The lobe measures 6.8 cm SI x 4.4 cm ML x 3.6 cm AP and the isthmus measures 2.2 SI x 0.7 cm ML x 0.8 cm AP. The external surfaces have fibrous adhesions. No parathyroid glands or lymph nodes are identified grossly. The external surface is painted with blue dye. The lobe is almost completely replaced by a delineated solid nodule that measures 5.1 cm SI x 4.3 cm ML x 3.5 cm AP. A small rim of compressed normal tissue is identified. Sections of nodule and normal tissue are stored frozen. The remainder of the specimen is submitted as follows:

1A-AA	lobe, superior to inferior
1AB-AC	isthmus

Microscopic Description

Surgical Pathology Consultation Report

The dominant tumor has central degeneration consistent with the site of previous biopsy; there is artefactual displacement of tumor into vascular channels at this site. The tumor is negative for nuclear transformation of beta-catenin; intact membranous staining is identified. Only scattered leukocytes stain for CD5.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed By: [Signature]	Date Reviewed: 6/12/12	